



**RFI Response: Federal Register Document
No. 2025-23641 (HHS Health Sector AI
RFI)**

Introduction

Design Interactive, LLC (DI) appreciates the opportunity to provide comments on the Department of Health and Human Services' Request for Information on accelerating the adoption and use of artificial intelligence (AI) as part of clinical care. The RFI identifies the importance of regulation, reimbursement, and research & development (R&D) levers in enabling safe, effective AI adoption. [orangeslices.ai], [vaoig.gov]

Our perspective is grounded in real-world experience deploying AI-enabled, machine-learning-powered systems inside large federal healthcare environments, particularly the U.S. Department of Veterans Affairs (VA). For example, DI has developed the AI-enabled competency development platform Nexora under as part of a Small Business Innovative Research (SBIR) contract. We align our comments explicitly with the VA Artificial Intelligence Workforce Resources Blueprint (2024) (a publicly available federal strategy that documents the organizational, workforce, governance, and risk-management challenges federal health systems face when adopting AI at scale). [federalregister.gov]

DI deploys AI-enabled adaptive content generation systems that support informaticists and clinical-facing operational staff by using multiple machine-learning models and adaptive/automated content generation to produce tailored microlearnings, job aids, and workflow-support materials. While the products are delivered as SaaS, they behave operationally as an AI system requiring:

- Data ingestion and structuring
- Model-based content generation
- AI lifecycle monitoring
- Human oversight
- Workflow integration

Barrier A: Governance & Decision Rights Become Unclear for AI Systems

General Barrier for AI Systems

The VA Artificial Intelligence (AI) Workforce Resources Blueprint highlights that AI systems require distinct governance roles such as AI model owners, risk stewards, data governance authorities, evaluation leads, and monitoring leads. These roles do not exist in legacy organizational structures. [federalregister.gov]

AI systems differ from traditional platforms because they introduce:

- Dynamic model behavior
- Need for model oversight
- Bias/drift monitoring
- Interpretability considerations
- Risk-tiering
- Human-in-the-loop safety requirements

Traditional “software owner” roles do not cover these responsibilities.

Capability Example

Because adaptive content-generation pipelines use dynamic machine-learning models and adaptive content-generation pipelines, their approval required decisions on:

- Who evaluates the AI components?
- Who owns AI-generated content risk?
- Who approves ongoing updates?
- Who monitors model performance or drift?

Inside VA, no single director or office could claim AI accountability, even when groups could claim other platform responsibility. This created slowdowns and repeat loops through disparate stakeholders. This problem is explicitly predicted by the VA Artificial Intelligence Workforce Resources Blueprint, which notes that AI governance must be cross-functional and cannot be assigned to a single traditional program office.

Recommendations to HHS

HHS should promote a federal reference architecture for AI governance, including:

- Defined AI model owners
- Risk-tiered approval requirements
- Required roles for evaluation, monitoring, and oversight
- Human-in-the-loop governance expectations

Barrier B: Funding Misalignment for AI Lifecycle Sustainment

General Barrier for AI Systems

AI systems are not “build once” tools. The VA Artificial Intelligence Workforce Resources Blueprint emphasizes that agencies must support:

- AI model lifecycle management
- Ongoing retraining
- Validation and re-validation
- Monitoring for drift and bias
- Workforce upskilling
- Continuous model governance

Capability Example

Operational AI platforms are often funded under mechanisms like SBIR Phase III, but:

- SBIR cannot fund model retraining,
- Nor AI monitoring,
- Nor algorithmic updates,
- Nor evaluation frameworks,
- Nor responsible-AI governance participation.

AI lifecycle tasks were not yet funded because VA had no dedicated AI adoption or sustainment funding mechanism. This is exactly the funding gap the VA Artificial Intelligence Workforce Resources Blueprint identifies. Agencies lack budget categories for AI sustainment, creating “pilot purgatory.”

Recommendations to HHS

HHS should create or encourage:

- Reimbursement codes or programmatic allowances for AI evaluation and monitoring

- Payment models that recognize AI lifecycle costs, not just acquisition
- Funding definitions that distinguish “software maintenance” from AI sustainment (drift monitoring, retraining, oversight)

This ensures health systems can responsibly maintain AI tools.

Barrier C: AI Hosting, Security, and Infrastructure Requirements Are Harder Than SaaS

General Barrier for AI Systems

The VA Artificial Intelligence Workforce Resources Blueprint identifies “AI-enabling infrastructure” (including compute, data platforms, monitoring pipelines, and secure environments for model evaluation) as essential for AI deployment. [federalregister.gov]

AI hosting involves factors SaaS does not:

- Access to training/evaluation datasets
- Secure environments for PHI-based model operation
- Integration into monitoring pipelines
- Support for retraining workflows
- Logging for auditability and oversight

Capability Example

For cloud-hosted AI systems, choosing between agency-hosted FedRAMP and commercial FedRAMP was not just an IT choice:

- Hosting choices determined access to model inputs
- They affected whether automated content generation could run in real time
- They determined if model-usage logs were available for governance
- They dictated alignment with Blueprint requirements for responsible AI lifecycle monitoring

This ambiguity slowed implementation and increased governance risk.

Recommendations to HHS

HHS should publish AI-specific hosting and ATO guidance, answering:

- When must AI run in agency-hosted FedRAMP?
- When can it run in external FedRAMP environments?
- What AI-specific ATO documentation is needed (model cards, drift monitoring plans, evaluation data)?
- How should AI logs and audit trails be handled?

This clarity would directly accelerate AI adoption across federal health systems.

Barrier D: Workforce Readiness Gaps

General Barrier for AI Systems

The Blueprint defines dozens of AI competencies required across federal healthcare workforces — including:

- AI literacy
- Understanding model limitations
- Ability to interpret AI outputs
- Ability to identify bias/drift
- Human-AI teaming skills
- Evaluation & monitoring competencies [federalregister.gov]

These competencies are not required for SaaS, making AI adoption uniquely dependent on workforce readiness.

Capability Example

Automated, adaptive content generation requires informaticists to understand:

- The models' intended use
- The boundaries of automated recommendations
- How to confirm accuracy
- How to escalate anomalies
- How their oversight mitigates risk

Traditional training for SaaS would not prepare them for this type of AI oversight.

Recommendations to HHS

HHS should:

- Promote AI workforce competency frameworks consistent with the Blueprint
- Support training for interpreting and evaluating AI outputs
- Encourage human-in-the-loop safety roles in clinical settings
- Fund workforce development aligned to AI risk-tiered use cases

Summary of Recommendations Aligned to HHS's Levers

Regulation

- Create federal AI governance role templates
- Standardize AI risk-tiering and evaluation expectations
- Provide AI-specific ATO, hosting, and security requirements

Reimbursement

- Support funding for AI lifecycle sustainment
- Create incentives or pathways that reward AI monitoring, evaluation, and clinician burden reduction

Research & Development

- Fund implementation science for AI integration
- Create AI evaluation toolkits and testbeds
- Support research into AI drift, monitoring, and human oversight models

Conclusion

The challenges DI experienced in deploying AI systems inside VA were not generic software obstacles, but rather the systemic AI adoption barriers documented in the VA AI Workforce Resources Blueprint (2024). [federalregister.gov]

These barriers demonstrate why HHS must address governance, workforce readiness, AI-specific infrastructure, and lifecycle sustainment to accelerate safe, effective AI adoption across the U.S. health system.

Design Interactive, LLC appreciates the opportunity to provide input and stands ready to support continued collaboration as HHS develops national frameworks for responsible AI in clinical care.